

DIABETMIN XR TABLETS

VIDIAxx-09 (MY HK)

DESCRIPTION

Diabetmin XR 750mg Tablets: 21x9mm, Oblong, white to off-white uncoated tablet with deep convex faces and "HOVID" embossed on one face.

Diabetmin XR 1000mg Tablets: 23x11mm, Oblong, white to off-white uncoated tablet, biconvex faces and "HOVID" embossed on one face.

COMPOSITION

Diabetmin XR 750mg Tablets: Metformin hydrochloride 750mg equivalent to 585mg of metformin base (Extended Release).

Diabetmin XR 1000mg Tablets: Metformin hydrochloride 1000mg equivalent to 780mg of metformin base (Extended Release).

ACTIONS AND PHARMACOLOGY

Metformin is a biguanide with antihyperglycaemic effects, lowering both basal and postprandial plasma glucose. It does not stimulate insulin secretion and therefore does not produce hypoglycaemia.

Metformin may act via 3 mechanisms:

- Reduction of hepatic glucose production by inhibiting gluconeogenesis and glycogenolysis
- In muscle, by increasing insulin sensitivity, improving peripheral glucose uptake and utilisation
- And, delay of intestinal glucose absorption Metformin stimulates intracellular glycogen synthesis by acting on glycogen synthase. Metformin increases the transport capacity of all types of membrane glucose transporters (GLUT).

PHARMACOKINETICS

Absorption

After an oral dose of the prolonged release tablet, metformin absorption is significantly delayed compared to the immediate release tablet with a Tmax at 7 hours (Tmax for the immediate release tablet is 2.5 hours). At steady state, similar to the immediate release formulation, Cmax and AUC are not proportionally increased to the administered dose. The AUC after a single oral administration of 2000 mg of metformin prolonged release tablets is similar to that observed after administration of 1000 mg of metformin immediate release tablets b.i.d. Intrasubject variability of Cmax and AUC of metformin prolonged release is comparable to that observed with metformin immediate release tablets. When the prolonged release tablet is administered in fasting conditions the AUC is decreased by 30% (both Cmax and Tmax are unaffected). Mean metformin absorption from the prolonged release formulation is almost not altered by meal composition. No accumulation is observed after repeated administration of up to 2000 mg of metformin as prolonged release tablets.

Distribution

Plasma protein binding is negligible. Metformin partitions into erythrocytes. The blood peak is lower than the plasma peak and appears at approximately the same time. The red blood cells most likely represent a secondary compartment of distribution. The mean Vd ranged between 63-276 L.

Metabolism

Metformin is excreted unchanged in the urine. No metabolites have been identified in humans.

Excretion

Renal clearance of metformin is > 400 ml/min, indicating that metformin is eliminated by glomerular filtration and tubular secretion. Following an oral dose, the apparent terminal elimination half-life is approximately 6.5 hours. When renal function is impaired, renal clearance is decreased in proportion to that of creatinine and thus the elimination half-life is prolonged, leading to increased levels of metformin in plasma.

Characteristics in specific groups of patients

Renal impairment

The available data in subjects with moderate renal insufficiency are scarce and no reliable estimation of the systemic exposure to metformin in this subgroup as compared to subjects with normal renal function could be made. Therefore, the dose adaptation should be made upon clinical efficacy/tolerability considerations.

INDICATION

Reduction in the risk or delay of the onset of type 2 diabetes mellitus in adult, overweight patients with Impaired Glucose Tolerance and/or Impaired Fasting Glucose , and/or increased HbA1C who are:

- at high risk for developing overt type 2 diabetes mellitus and
- still progressing towards type 2 diabetes mellitus despite implementation of intensive lifestyle change for 3 to 6 months

Treatment with Diabetmin XR must be based on a risk score incorporating appropriate measures of glycaemic control and including evidence of high cardiovascular risk.

Lifestyle modifications should be continued when metformin is initiated, unless the patient is unable to do so because of medical reasons.

Treatment of type 2 diabetes mellitus in adults, when dietary management and exercise alone does not result in adequate glycaemic control. Diabetmin XR may be used as monotherapy or in combination with other oral antidiabetic agents, or with insulin.

For Diabetmin XR 750mg Tablets: A reduction of diabetic complications has been shown in overweight type 2 diabetic patients treated with immediate release metformin as first-line therapy after diet failure.

CONTRAINDICATIONS

- Hypersensitivity to metformin or to any of the excipients
- Diabetic pre-coma
- Severely reduced kidney function (GFR <30 mL/min)
- Any type of acute metabolic acidosis (such as lactic acidosis, diabetic ketoacidosis)
- Acute conditions with the potential to alter renal function such as:
 - dehydration,
 - severe infection,
 - shock
- Disease which may cause tissue hypoxia (especially acute disease, or worsening of chronic disease) such as:
 - decompensated heart failure,
 - respiratory failure,
 - recent myocardial infarction,
 - shock
- Hepatic insufficiency, acute alcohol intoxication, alcoholism

WARNINGS AND PRECAUTIONS

LACTIC ACIDOSIS

Lactic acidosis is a very rare, but serious, metabolic complication, most often occurs at acute worsening of renal function or cardiorespiratory illness or sepsis. Metformin accumulation occurs at acute worsening of renal function and increases the risk of lactic acidosis.

In case of dehydration (severe diarrhoea or vomiting, fever or reduced fluid intake), metformin should be temporarily discontinued and contact with a health care professional is recommended.

Medicinal products that can acutely impair renal function (such as antihypertensives, diuretics and NSAIDs) should be initiated with caution in metformin-treated patients. Other risk factors for lactic acidosis are excessive alcohol intake, hepatic insufficiency, inadequately controlled diabetes, ketosis, prolonged fasting and any conditions associated with hypoxia, as well as concomitant use of medicinal products that may cause lactic acidosis.

Patients and/or care-givers should be informed of the risk of lactic acidosis. Lactic acidosis is characterised by acidotic dyspnoea, abdominal pain, muscle cramps, asthenia and hypothermia followed by coma. In case of suspected symptoms, the patients should stop taking metformin and seek immediate medical attention. Diagnostic laboratory findings are decreased blood pH (< 7.35), increased plasma lactate levels (>5 mmol/L) and an increased anion gap and lactate/pyruvate ratio.

Renal function

A GFR should be assessed before treatment initiation and regularly thereafter (See section Dosage and Administration). Metformin is contraindicated in patients with GFR < 30 mL/min and should be temporarily discontinued in the presence of conditions that alter renal functions (See section Contraindications).

Cardiac function

Patients with heart failure are more at risk of hypoxia and renal insufficiency. In patients with stable chronic heart failure, metformin may be used with a regular monitoring of cardiac and renal function. For patients with acute and unstable heart failure, metformin is contraindicated.

Elderly

Due to the limited therapeutic efficacy data in the reduction of risk or delay of type 2 diabetes in patients 75 years and older, metformin initiation is not recommended in these patients.

Administration of iodinated contrast media

Intravascular administration of iodinated contrast agents may lead to contrast induced nephropathy, resulting in metformin accumulation and an increased risk of lactic acidosis. Metformin should be discontinued prior to or at the time of the imaging procedure and not restarted until at least 48 hours after, provided that renal function has been re-evaluated and found to be stable.

Surgery

Metformin must be discontinued at the time of surgery under general, spinal or epidural anaesthesia. Therapy may be restarted no earlier than 48 hours following surgery or resumption of oral nutrition and provided that renal function has been re-evaluated and found to be stable.

Other precautions

All patients should continue their diet with a regular distribution of carbohydrate intake during the day. Overweight patients should continue their energy-restricted diet. The usual laboratory tests for diabetes monitoring should be performed regularly. Metformin alone never causes hypoglycaemia, although caution is advised when it is used in combination with insulin or other oral antidiabetics (e.g. sulphonylureas or meglitinides).

PREGNANCY AND LACTATION

Pregnancy

Uncontrolled diabetes during pregnancy (gestational or permanent) is associated with increased risk of congenital abnormalities and perinatal mortality. When the patient plans to become pregnant and during pregnancy, it is recommended that impaired glycaemic control or diabetes are not treated with metformin. For diabetes it is recommended that insulin should be used to maintain blood glucose levels as close to normal as possible to reduce the risk of malformations of the foetus.



Lactation

Metformin is excreted into human breast milk. No adverse effects were observed in breastfed newborns/infants. However, as only limited data are available, breastfeeding is not recommended during metformin treatment. A decision on whether to discontinue breast-feeding should be made, taking into account the benefit of breast-feeding and the potential risk to adverse effect on the child.

Fertility

Fertility of male or female rats was unaffected by metformin when administered at doses as high as 600 mg/kg/day, which is approximately three times the maximum recommended human daily dose based on body surface area comparisons.

DRUG INTERACTIONS

Concomitant use not recommended

Alcohol

Acute alcohol intoxication is associated with an increased risk of lactic acidosis, particularly in case of fasting, malnutrition or hepatic impairment.

Iodinated contrast media

Metformin must be discontinued prior to or at the time of the imaging procedure and not restarted until at least 48 hours after, provided that renal function has been re-evaluated and found to be stable.

Combinations requiring precautions for use

Some medicinal products can adversely affect renal function which may increase the risk of lactic acidosis, e.g. NSAIDs, including selective cyclo-oxygenase (COX) II inhibitors, ACE inhibitor, angiotensin II receptor antagonists and diuretics, especially loop diuretics. When starting or using such products in combination with metformin, close monitoring of renal function is necessary.

Medicinal products with intrinsic hyperglycaemic activity (e.g. glucocorticoids (systemic and local routes) and sympathomimetics). More frequent blood glucose monitoring may be required, especially at the beginning of treatment. If necessary, adjust the metformin dosage during therapy with the other drug and upon its discontinuation.

Organic cation transporters (OCT)

Metformin is a substrate of both transporters OCT1 and OCT2.

Co-administration of metformin with

- Inhibitors of OCT1 (such as verapamil) may reduce efficacy of metformin.
- Inducers of OCT1 (such as rifampicin) may increase gastrointestinal absorption and efficacy of metformin.
- Inhibitors of OCT2 (such as cimetidine, dolutegrivir, ranolazine, trimethoprim, vandetanib, isavuconazole) may decrease the renal elimination of metformin and this lead to an increase in metformin plasma concentration.
- Inhibitors of both OCT1 and OCT2 (such as crizotinib, olaparib) may alter efficacy and renal elimination of metformin.

Caution is therefore advised, especially in patients with renal impairment, when these drugs are co-administered with metformin, as metformin plasma concentration may increase. If needed, dose adjustment of metformin may be considered as OCT inhibitors/inducers may alter the efficacy of metformin.

MAIN SIDE/ADVERSE EFFECTS

Metabolism and nutrition disorders

Very rare:

- Lactic acidosis
- Decrease of vitamin B12 absorption with decrease of serum levels during long-term use of metformin. Consideration of such an aetiology is recommended if a patient presents with megaloblastic anaemia.

Nervous system disorders

Common: Taste disturbance

Gastrointestinal disorders

Very common: Gastrointestinal disorders such as nausea, vomiting, diarrhoea, abdominal pain and loss of appetite. These undesirable effects occur most frequently during initiation of therapy and resolve spontaneously in most cases. A slow increase of the dose may also improve gastrointestinal tolerability.

Hepatobiliary disorders

Very rare: Isolated reports of liver function tests abnormalities or hepatitis resolving upon metformin discontinuation.

Skin and subcutaneous tissue disorders

Very rare: Skin reactions such as erythema, pruritus, urticaria.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

Metformin monotherapy does not cause hypoglycaemia and therefore has no effect on the ability to drive or to use machines. However, patients should be alerted to the risk of hypoglycaemia when metformin is used in combination with other antidiabetic agents (eg. sulphonylureas, insulin or meglinitides).

OVERDOSE AND TREATMENT

Hypoglycaemia has not been seen with metformin doses of up to 85 g, although lactic acidosis has occurred in such circumstances. High overdose or concomitant risks of metformin may lead to lactic acidosis. Lactic acidosis is a medical emergency and must be treated in hospital. The most effective method to remove lactate and metformin is haemodialysis.

DOSAGE AND ADMINISTRATION

Adults with normal renal function (GFR≥ 90 mL/min)

Reduction in the risk or delay of the onset of type 2 diabetes:

- Metformin should only be considered where intensive lifestyle modifications for 3 to 6 months have not resulted in adequate glycaemic control.

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- The therapy should be initiated with one tablet metformin hydrochloride extended release 500 mg once daily with the evening meal.
- After 10 to 15 days dose adjustment on the basis of blood glucose measurements is recommended (OGTT and/or FPG and/or HbA1c values to be within the normal range). A slow increase of dose may improve gastro-intestinal tolerability. The maximum recommended dose is 2000 mg once daily with the evening meal.
- It is recommended to regularly monitor (every 3-6 months) the glycaemic status (OGTT and/or FPG and/or HbA1c value) as well as the risk factors to evaluate whether treatment needs to be continued, modified or discontinued.
- A decision to re-evaluate therapy is also required if the patient subsequently implements improvements to diet and/or exercise, or if changes to the medical condition will allow increased lifestyle interventions to be possible.

Monotherapy or combination with other oral antidiabetic agents:

- For patients new to metformin hydrochloride, the usual starting dose of Diabetmin XR is 500 mg or 750 mg once daily given with the evening meal. After 10 to 15 days the dose should be adjusted on the basis of blood glucose measurements. A slow increment in dose may improve gastrointestinal tolerability. The maximum recommended dose is 2000mg once daily with the evening meal.
- In patients already treated with metformin tablets, the starting dose of Diabetmin XR should be equivalent to the daily dose of metformin immediate release tablets. In patients treated with metformin hydrochloride at a dose above 2000 mg daily, switching to Diabetmin XR is not recommended.
- If glycaemic control is not achieved on 2000 mg metformin hydrochloride extended release once daily, patients may be switched to standard metformin hydrochloride immediate-release to a maximum dose of 3000 mg daily.
- If transfer from another oral antidiabetic agent is intended: discontinue the other agent and initiate Diabetmin XR at the dose indicated above.

Combination with insulin

Metformin hydrochloride and insulin may be used in combination therapy to achieve better blood glucose control. The usual starting dose of metformin hydrochloride extended release is one tablet 500 mg or 750 mg once daily with the evening meal, while insulin dosage is adjusted on the basis of blood glucose measurements. After titration, switch to Diabetmin XR 1000 mg should be considered.

Elderly

Due to the potential for decreased renal function in elderly subjects, the metformin dosage should be adjusted based on renal function. Regular assessment of renal function is necessary (see Special warnings and special precautions for use).

Benefit in the reduction of risk or delay of the onset of type 2 diabetes mellitus has not been established in patients 75 years and older (see section Pharmacodynamic properties) and metformin initiation is therefore not recommended in these patients.

Renal impairment

A GFR should be assessed before initiation of treatment with metformin containing products and at least annually thereafter. In patients at an increased risk of further progression of renal impairment and in the elderly, renal function should be assessed more frequently, e.g. every 3-6 months.

GFR mL/min	Total maximum daily dose	Additional Considerations
60 - 89	2000 mg	Dose reduction may be considered in relation to declining renal function
45 – 59	2000 mg	Factors that may increase the risk of lactic acidosis should be reviewed before considering initiation of metformin. The starting dose is at most half of the maximum dose.
30 – 44	1000 mg	
<30	-	Metformin is contraindicated.

Paediatric population

In the absence of available data, Diabetmin XR should not be used in children.

For oral use.

Note: The information given here is limited. For further information, kindly consult your doctor or pharmacist.

Storage: Store below 30°C. Protect from light and moisture.

Presentation/ Packing: Blister pack of 30's, 60's and 100's.

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